Medical Student Guide: Outpatient Child & Adolescent Psychiatry

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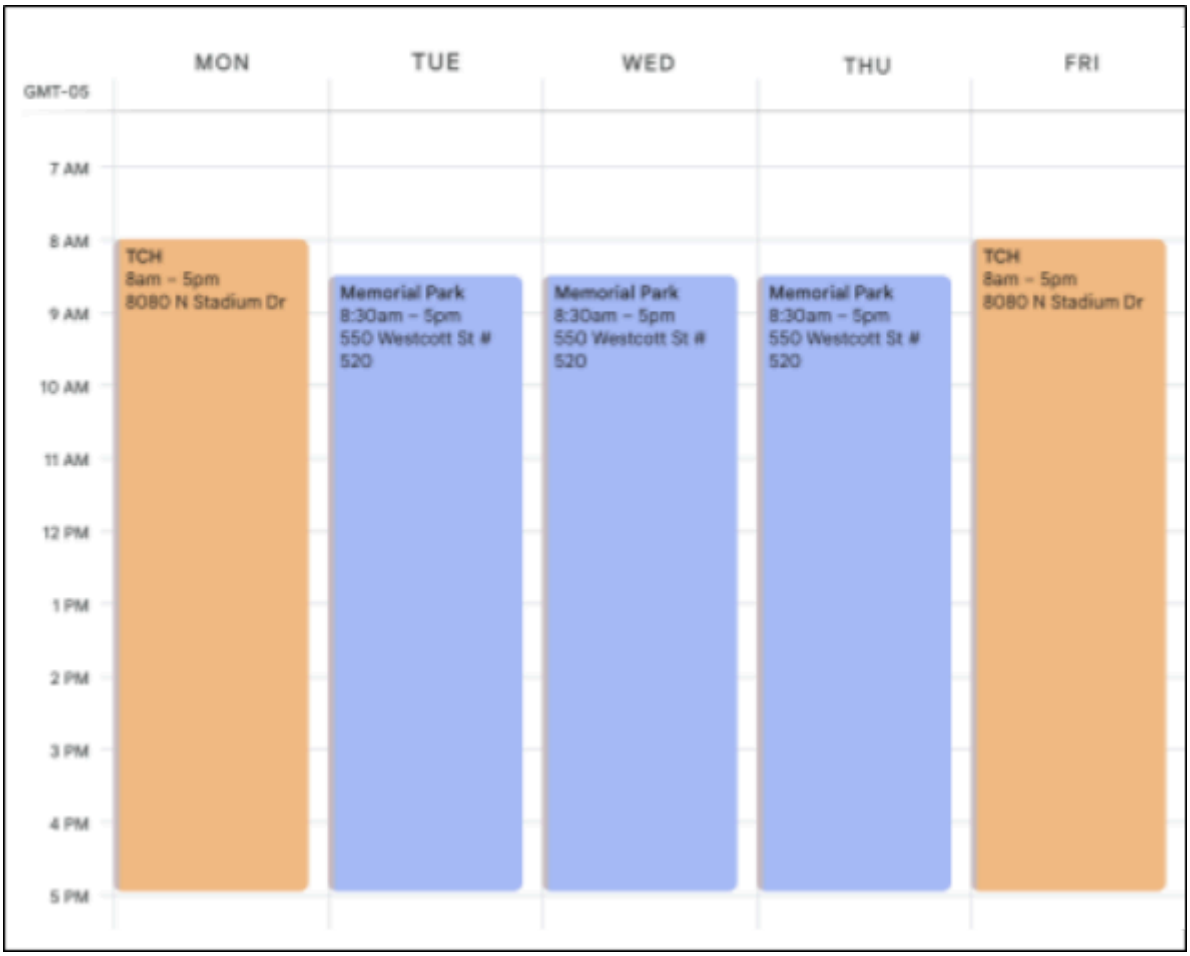
Overview

Welcome to Outpatient Child & Adolescent Psychiatry! You will be exposed to a variety of pediatric psychiatric conditions and gain hands-on experience in assessment and management.

During your rotation, please bring a personal laptop so you can access the EMR.

You will rotate at two clinics: TCH Psychiatry and Memorial Park Psychiatry (please find more information on each clinic below). Below you will find the weekly schedule.



TCH Psychiatry

Address: 8080 N Stadium Drive

Parking: Free, you can park anywhere on the right side of the lot that is not marked as reserved.

Locating the clinic:

1. Enter the building, then enter the first door on the left.
2. Once in the waiting room, kindly ask the front desk to let you inside.
3. Enter the first door on your left.
4. Walk to the end of the hallway, then turn left.
5. The last door on the right is Dr. Mao's office. If Dr. Mao is not in her office, then try to locate the residents/fellows who are working with her for the day. They are normally in one of the patient rooms chart reviewing.

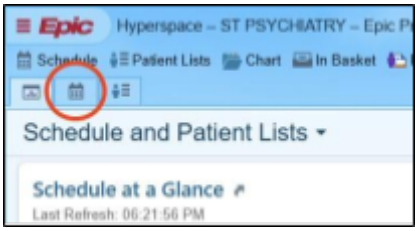
What to expect: You will follow a resident/fellow who will then staff with Dr. Mao. Some will have you conduct interviews, write notes, and perform the mental status exam (MSE).

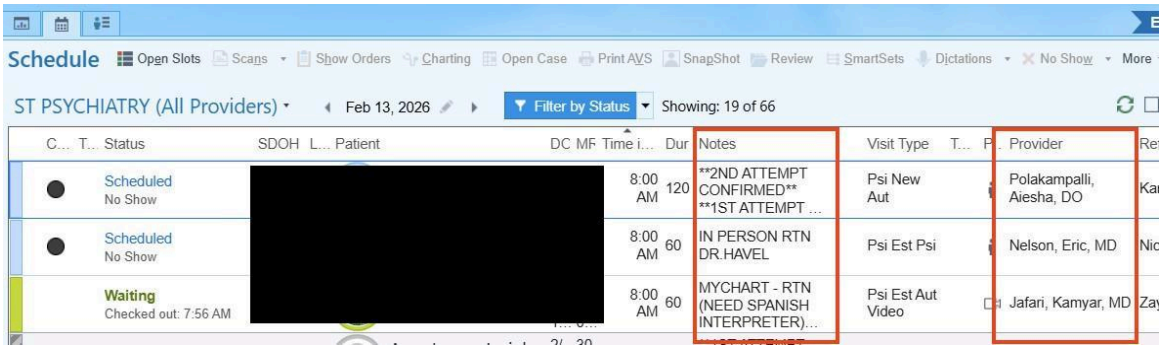
Others will just have you shadow and present to Dr. Mao.

Wifi: Select TCH then log into your TCH account.

EMR:

1. Log onto Epic and select the context as “ST Psychiatry.”
2. Hover over the column “Notes” to see which patients will be staffed with Dr. Mao. The column “Provider” will be the name of the resident/fellow.
3. Click “ST Psychiatry (All Providers)” to select the name of the resident/fellow you are working with for the day.





Memorial Park Psychiatry

Address: 550 Westcott, Suite #520

Parking: Free, you can park anywhere that is not marked as reserved either in front of the building or in the parking garage below.

Locating the clinic:

1. Enter the building, then take the elevators to the fifth floor.
2. When you come out of the elevators, the clinic is through the door on the right.
3. Once in the waiting room, enter through the door on the left. If the door is not open, lightly tap on the glass and ask the staff to let you in.
4. Walk down the hallway until you see Dr. Mao’s office on the right side of the hallway.

What to expect: You will get to observe and help Dr. Mao conduct interviews with patients. When observing, you will have the opportunity to write notes and place orders. When seeing patients on your own, you will present them to Dr. Mao and work on establishing

plans.

Wifi: Memorial Work, password: memorial 520 (space in between)

EMR: Memorial Park primarily uses Insync as an EMR but you may need to access records from Valant (the previous EMR). You can find the links/accounts for both below. Once logged into Insync, you will see the schedule and can follow the instructions below to navigate the chart. If asked for an authentication code when logging in, ask the front desk for the code.

Login info:

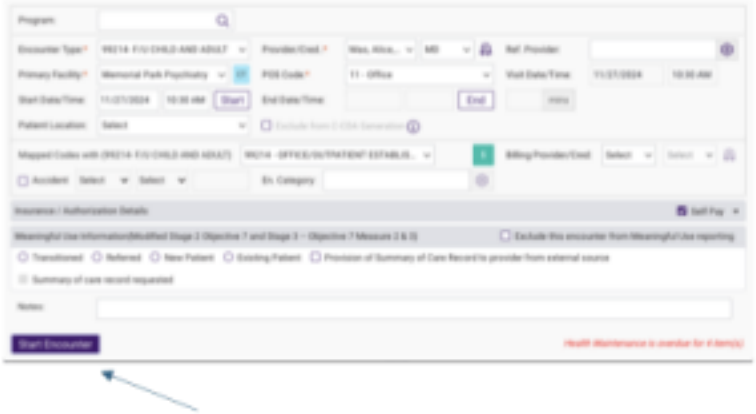
	Link Username	Password
Insync *New EMR*	https://beaconoutpatient.insynchcs.com/?UserLogout=True User: Amao	Memorial520!
Valant *Old EMR*	https://www.valant.io/io/Account/LogOn?ReturnUrl=%2Fio%2FClient User: bramsmaooffice@aol.com	Nathan520!

Doxy.me
This is the site to join telehealth visits.
Login info:
Website: https://doxy.me/en/
User: maobrams@aol.com Pass: Memorial1

Getting Started in Insync:

1. Click on this icon on the left-hand side to find the patient list:
2. Click on the patient's name and click “Start encounter” or “encounter in progress.”

3. Click “start encounter” on the bottom left of the pop up window.



4. If the patient has a pre-existing note in the system, it will give you the option to **"Copy/Replace"** on the bottom left of the screen. Click this to automatically start a note for the visit based on the previous note.
    a. If not, go to Valant and search for the patient in "Open Chart." When searching the patient in Valant.io, it's easiest to search by birthdate or the first three letters of the last name and first three letters of the first name (Ex: John Smith= SMIJOH)
    b. Next, click documentaries and find their most recent "Psychiatric Progress Note"

    c. Copy and paste the "Impression" section from Valant note into both the HPI and Impression/Plan into new Insyncs note. Please see below where each

section is:

5. Before each patient, divide these tasks among the students: HPI, Impression/Plan, MSE, Meds, psychotherapy. This is the part of the chart each student is responsible for during

the encounter, see descriptions below.

a. **HPI:** Fill in interval history and make updates to problems, ensuring to write the patient's rating for mood/anxiety. Be sure to include names of all students involved.

- Student who wrote the HPI should have an asterisk after their name*.
- Student who wrote the plan should receive two asterisks after their name**.
- Student who conducted the MSE should receive_MSE after their name.
- Student who wrote the psychotherapy note should have a tilde (~) after their name.
- Student who updated medications should have a caret after their name^.
    i. Example: medical students Tom Jones*, Billy Gonzalez**, Mike Smith_MSE, Leah Shah~, and Sophia Li^.

*** HPI is the only section that does not save automatically. Click save!*

b. **Impression/Plan:** NO interval history, can have a sentence or two but really that goes in HPI or supportive psychotherapy. Focus on problem based; start by noting current dosage of medication; note any positive and negative ROS. Discuss if the patient wants to go up, down, or stay stable on dose. Conclude with summary of changes. Make sure to add the following sentence above the medication list if not already there: "I serve as the patient's ongoing psychiatrist coordinating BH care; today's plan continues longitudinal management and care coordination."

i. Example:
I serve as the patient's ongoing psychiatrist coordinating BH care; today's plan continues longitudinal management and care coordination.

Plan

- Increase Fluoxetine 10mg PO qD to 20mg PO qD for depression.
- Continue Risperdal 0.5mg PO TID for irritability.
- Return to clinic in 2.5 months.

c. **MSE:** You will perform the MSE. Then, underneath the MSE tab, there is a text box on the right side for each item. Use that to specify what you did to assess the item. (Ie. Concentration intact to serial 7s. Attention intact to spelling world forwards and backwards.) Check PHQ-9 and GAD-7 scores in the Assessment Tools tab and complete with the patient if not already done prior to appointment.

d. **Meds:**

1. Click on "Medications/Allergies" tab.
2. Click "Manage."
3. Verify the patient's pharmacy by clicking the house icon on the top right corner:

4. Select with the check box which meds you want to renew, then click **instant renewal.**
5. "X" out of the window (hit Yes if it asks you are you sure)
6. Click "Prescribe". This is where you will check the meds before transmitting.

7. For controlled substances, please see below:
    - Controlled substances (stimulants, some sleep meds) can only be prescribed in 30 day quantities at a time, but follow up visits are usually every 2.5 months. Therefore, the patient needs three separate 30 day prescriptions of the stimulant. To date these prescriptions, you need to put a “do not fill date.” For the first prescription ask the patient when they last refilled.
    - IF they say they are just about out and need more, use “do not fill date” of TODAY, for the first one. For each prescription after, go 30 days forward and 2 days back. So if the first “do not fill date” is July 7, the next would be August 5, and final would be Sept 3. The “do not fill date” means they cannot fill that prescription until the date, but starting then they have 20 days to pick it up.
    - IF they say they just refilled their meds right before this visit, then you would only send two 30 day prescriptions of stimulant (2 separate prescriptions with first one being about 30-2=28 days from patient's recent refill). You will need to have Dr. Mao give you a six digit code, which after clicking “Transmit Rx”, you will see a space to enter the code (“OTP code”).

d. **Psychotherapy:** Go to “Plan” then select “Supportive psychotherapy.”

- Write in: “Supportive psychotherapy provided for over 16 minutes.” Then describe in *broad strokes* what Dr. Mao and the patient discussed. These are the life updates and things that are going on that you can follow up in the next visit.
- Example: Patient is currently going through a difficult time with a divorce. Patient has also lost their job and is in the process of searching for a new job. Discussed coping strategies and psychotherapy to deal with current stressors.

Interviewing the Patient

For established patients:

- Ask about how things are going and if anything has changed in their medical history -Confirm current medications, ensure they aren’t taking any new medications. If patients are prescribed PRNs, ask how often they end up needing to take it. For example, you will want to know if a patient is needing to take Xanax PRN often, because that means you probably need to change their scheduled med.
- Ask how they think this regimen is going and if they feel like any changes need to be made. Ask them to rate their mood on a scale of 1-10, with 1 being very sad and 10 being very happy. Ask them to rate their anxiety on a scale of 1-10, with 0 being no anxiety and 10 being very anxious.
- Ask how many hours of sleep they are getting a night and how their appetite is - Ask about any SI/HI, AVH, delusions, paranoia. Ask about side effects, for example: - SSRIs at initiation: headache, nausea, vomiting. Later, changes in libido, sexual function, weight,

- Stimulants: irritability, palpitations, suppressed appetite
- Antipsychotics: EPS, weight gain (esp for olanzapine), akathisia (esp for Abilify) -Confirm if they need refills. If they are taking controlled substances, need to ask when they last filled their prescription

*For patients with autism, ask the patient/parent how they are tolerating regimen/any recent changes in regimen. Perform a mental status exam that is appropriate for their level of engagement (see MSE section for more information)

For new patients:

- Start broad: "What brings you in today?" and tease apart symptoms to build differential -If diagnosed with psychiatric conditions in the past, then ask:
  - When were you first diagnosed with [condition]
  - Previous medications and response to them
  - Any side effects from prior medications
- Prior psychiatric hospitalizations, suicide attempts
- Medical, surgical, family history (including psychiatric history in the family)

If time, you can ask patients about social history:

Childhood	Tell me about your childhood? Where are you from? Did you need any accommodations in school? Did your mom have any issues with her pregnancy with you? Were you born at full term? Any issues developmentally? What did your parents do for work? Are they still with us? What is the highest level of education you have?
Family/Relationships	Are you currently married/dating? How long? Have you ever been divorced?
	Do you have any kids? If so, how old? Do you feel like you have friends that support you? Are you sexually active? Are you interested in men/women/both?
Work/School/Finances	What do you do for work/what are you studying in school? How would you describe your stress levels? Do you have any financial stress?
Substance use	For each, ask about duration and frequency: - Caffeine - Alcohol - Tobacco

	- Recreational drugs
Trauma	Have you experienced or witnessed any traumatic events (e.g. abuse, assault, neglect, witnessing violence)? Do you feel comfortable talking about past trauma? Have you had any significant losses in your life? Have you had any attempts to end your life?
Legal history	Do you currently or have you ever had any issues with the law? Have you ever been arrested?

Hobbies What do you like to do in your free time? What activities do you like to do to relax?

Mental Status Exam

- Orientation
  - What is your full name?
  - What day of the week and year is it?
  - Where are you right now?
- 3 items to remember (can be any 3 times): apple, pen, chair
  - Tell patient to repeat 3 words back to you and remember them, because they will repeat them in 5 minutes
- Spell "WORLD" forwards and backwards
- Numbers (you can do any set)
  - Tell patient to repeat any set of numbers forward: 3-5, 5-2-7, 4-1-9-2, 6-2-4-5-8, 7-3-9-1-4-8
  - Tell patient to repeat set of numbers backwards: 7-3, 4-5-7, 2-1-5-3
- Serial 7s
  - Tell patient to subtract 7 from 100 and continue subtracting 7 (stop at 51)
    - If struggling, subtract 3 from 20 and continue
    - If struggling, count from 20 backwards
- What is similar between these two objects?
  - An apple and an orange
  - A watch and a ruler
- What does this phrase mean to you?
  - Don't judge a book by its cover
  - The grass is always greener on the other side
- What would you do in this situation?
  - You see a letter with an address and a stamp in the middle of the street.
  - You see and smell smoke in the middle of a movie in a theatre.
- Repeat after me: no ifs, ands, or butts.
- Who is the president right now? And the 4 before him?
- Tell me the 3 items from before (apple, pen, chair)

- If patient is 65+ y.o. or at risk for dementia:
  - Ask them to repeat as many words as they can that start with "F" in 1 min.

If a patient is nonverbal and developmentally delayed, ask them to point at their nose, ears, other common body parts (do so without gesturing yourself). Can ask them to say ABCs and

count to 10, and identify colors. If they do well with this, then you can step up to a more formal mental status exam.

Presentations

When presenting, if there is time, you should try to follow the “formal” presentation format. Sometimes, usually at TCH clinic, you may defer to a “shortened” presentation to save time. Please find templates of each below:

Formal presentation

One Liner: This is [patient name], who is a [age] [gender] with a history of [psychiatric conditions].

HPI: Last visit we [insert any prior changes to medication regimens or any other pertinent information].

For their [condition], they are taking [insert medication and dosing].

They feel like medication [is/is not working].

[Describe current symptoms, mood rating].

Denies [side effects from medication, e.g. changes in appetite, sleep, etc.]

***MSE*:** On mental status exam…

Appearance: They appeared [their stated age, well groomed, appropriately dressed for the setting, disheveled, etc.]

Speech: Their speech is [normal, slow, rapid], and [pressured/non pressured]. **Mood:** Mood is described as [euthymic, anxious, depressed, etc.], and affect is [full range, restricted, blunted, etc.], which is

Short Presentation

One Liner: This is [patient name], who is a [age] [gender] with a history of [psychiatric conditions].

HPI: Last visit we [insert any prior changes to medication regimens or any other pertinent information].

For their [condition], they are taking [insert medication and dosing].

They feel like medication [is/is not working].

[Describe current symptoms, mood rating].

Denies [side effects from medication, e.g. changes in appetite, sleep, etc.].

I want to make [this change to their medication]

If you want to include a mental status exam, this is a shortened version for a presentation:

***MSE*:** On mental status exam…

- Patient is oriented x1/2/3/4
- Immediate memory and delayed recall [is/is not] preserved with

[congruent/incongruent] with stated mood.
[successful/unsuccessful] recall on
Thought Process: Thought process is [linear
digit span test and repeating words at
and goal-directed, tangential, circumstantial,
0 and 5 minutes
flight of ideas, loose associations].

- Concentration [intact/not intact] with

Thought Content: Thought content
world forward and backwards
[does/does not] have [auditory or visual

- Attention [intact/not intact] with serial

hallucinations, delusions, SI/HI].
7s.
Cognition: (*always explain the error if*

- Patient has a [strong/limited] fund of

present) On cognitive exam, the patient is:
knowledge, demonstrated

- **Orientation:** Oriented to [person,

[appropriate/inappropriate]
place, time, and event].
abstraction, and exhibits

- **Memory:** Immediate recall [is/is not]

[intact/impaired] judgement.
intact as demonstrated by
[successful/unsuccessful]

- **Attention:** Attention [is/is not] intact as evidenced by [accurate/inaccurate] performance on serial 7s.
- **Concentration:** Concentration [is/is not] intact with [no errors/errors] in spelling [word] forward and backwards.
- **Abstract thinking:** Demonstrates [appropriate/inappropriate] abstraction when asked what is the similarity between [object] and [object]
- **Judgement:** [Exhibits/does not exhibit] intact judgment when asking about [scenario].

This MSE suggests [intact/impaired] cognitive function with deficits in [areas].

Assessment/Plan: Discuss your assessment (if the symptoms are

4P Model

Typically for initial intake visits, a section is included before the plan called “biopsychosocial formulation” which uses the 4Ps. These are predisposing, precipitating, perpetuating, and protective factors. The 4 Ps framework is based on the biopsychosocial model, which emphasizes the interaction of biological, psychological, and social factors in mental health. By considering these factors, clinicians can develop a comprehensive understanding of a patient's mental health condition and tailor effective treatment plans. Here is a list of factors you may consider:

4P Factor Model	Biopsychosocial Approach		
	Biological	Psychological	Social
Predisposing	Genetic vulnerability, toxic exposure in utero, birth complications, traumatic brain injury	Attachment style, personality traits, isolation, insecurities, fear of abandonment since childhood	Domestic violence, poverty and adversity, unstable home life, divorce
Precipitating	Iatrogenic reaction, poor sleep, substance use/misuse	Recent loss, stress, reexperience abandonment/fears	School stressors, loss of significant relationship, loss of home
Perpetuating	Poor response to medication, chronic illness/pain	Personality traits, coping mechanism, beliefs of self, others and the world	Role of stigma to access treatment, poor finance, ongoing transition
Protective	Adequate diet, sleep, good genes, physical exercise, resilience, intelligence	Insightful and cognitive behaviour strategies, coping skills, psychologically minded	Community, family and faith support, financial or disability support, GP support

Example:
A patient with depression may have…

- Predisposing factors: A family history of depression and a genetic predisposition to anxiety
- Precipitating factors: The recent loss of a job and a stressful relationship
- Perpetuating factors: Negative thoughts and beliefs about themselves, avoidance of social activities, and substance use
- Protective factors: A strong support system from family and friends, a history of resilience, and access to therapy

Resources

Benioff Children's Hospitals Provider Resources (Guidelines for screening and management for MDD, GAD, ADHD, Suicide): https://capp.ucsf.edu/ProviderResourcesv2

ADHD Medication Guide:
https://www.chconline.org/resourcelibrary/the-adhd-medication-guide-downloadable/

PsychDB (like UptoDate for Psychiatry, great for medical students): https://www.psychdb.com/

Overview of 4 Ps:
https://www.psychdb.com/teaching/biopsychosocial-case-formulation#the-formulation-table

MSE: https://www.psychdb.com/teaching/mental-status-exam-mse